



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : صباح مسعد سليمان برسوم

العنوان : عزبة النخل المرج
القاهرة

الحي / المدينة : المرج

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 05-10-2026

رقم البطاقة : 27210090100323 رقم التليفون : 01172236521

العمر (تاريخ الميلاد) : 09-10-1972 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الاورام

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي
الاورام المنتشر [14456]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صباح مسعد سليمان برسوم National ID: 27210090100323	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٢/٠٨ ٢٧٢١٠٠٩٠١٠٠٣٢٣
حاصلة على دبلوم ثانوي فني تجاري



أنش مميحية
عماد سمير لبيب جرجس
البطاقة منارية حتى ٢٠٢٩/٠٨/٢٣



(Dms)

من يد مدونة في - والأفصا
المودا

تم التوقيع بشار الحلي
سلام السيد

جمهورية مصر العربية
بطاقة تحقيق الشخصية



صباح

مهند سليمان برسوم

١٠ ش مكة المكرمة - عزبة النخل

المرج - القاهرة

١٩٧٢/١/١٩

٢٧٢١٠٠٩٠١٠٠٣٢٣

JN1370128



Ministry of Health
Kingdom of Saudi Arabia

Ministry of Health
Kingdom of Saudi Arabia



A-Patient ID:

1- Name: **مهاج عبد السلام بن محمد**

Sex:

MDT form

3- Medical number:

Female

63153479

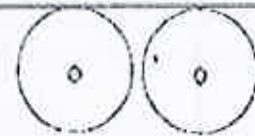
Q3 Test:

Q3 Test:

27/7/2023

2- ID number:

Patient Age: 52 Y



Family history:

Family history:

Phone:

Phone:

01225268734

EMR:

EMR:

Reason of MDT:

Reason of MDT:

Marital status:

Marital status:

HRS Score:

HRS Score:

GROUP:

GROUP:

Date of MDT:

Date of MDT:

2025-03-04

Comorbidity:

Comorbidity:

Menopausal status:

Menopausal status:

Initial stage:

Initial stage:

Clinical data

Breast Imaging:

Mets Workup:

DX:

ER:

PR:

Her 2 Neu:

SISH:

Ki67:

2021 LT BREAST
6/2023 SCIN

8/8

8/8

7/8

0/8

3

3

T:

N:

M:

1) BONE, LN

Sonomammography

PATIENT HISTORY

3/2021-->LEFT BREAST LOCALLY ADVANCED-->UPFRONT SM+SLNB-->T1CN0-->RECEIVED ADJ TAMOXIFEN FROM 5/2021 TILL 6/2023

CTCAP-->6/2023-->ENLARGED PATHOLOGICAL AXILNS+INTERNAL MAMMARY +SUPRACLAV BONE SCAN 6/2023-->BONE METS,RT HILAR AND OSEEIOUS LYTIC LESIONS

BX6/2023-->FROM SUPRACLAV-->METS ADENOCARCINOMA-->ER 8/8 PR 0/8 HER2 +3

PT RECEIVED PALLIATIVE RTH ON STERNAL BONE

REVISION OF BLOCKS= ER=8/8, PR=0/8,HER2+3

palliative RTH

MDT:taxol/herceptin +zometa till max response-->shifted to aramdix

SHE REPORTED INCREASE STERNAL PAIN WHEN STOPPED HERCEPTIN FOR 2 MN

maintance herceptin x24 cycle+femara-->multiple enlarged mediastinal LN

CT CAP 2/2025:NEWLY DEVELOPED SUBCRINAL LN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR tykerb /xeloda

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Name:	صباح مسعد سليمان	
MR. :	2907	
Detailed Medical Report		
		السيد
تخية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient presented for administration of oral capecitabine (Xeloda) at a dose schedule of $3 \times 2 \times 14$ and intravenous trastuzumab (Tykerb) after laboratory evaluation; the labs were acceptable for the addition of zoledronic acid (Zometa), and the patient was cleared to receive the planned therapy. The patient returned for capecitabine (Xeloda) at a schedule of $3 + 3 \times 14$ and trastuzumab (Tykerb) at a schedule of $5 \times 1 \times 30$; laboratory results were accepted and a computed tomography of the chest, abdomen, and pelvis performed on 7 April 2026 demonstrated a few enlarged mediastinal lymph nodes, the largest measuring 9×5 mm, pending radiologist comparison. A follow-up note documented that the CT CAP performed in April 2026 showed stable disease; the patient continued on capecitabine (Xeloda) and trastuzumab (Tykerb) with the next clinic visit scheduled for 23 August 2026. The patient's oncologic history includes a left breast locally advanced carcinoma diagnosed in March 2021, treated with upfront surgery (mastectomy and sentinel lymph node biopsy) revealing a T1C N0 tumor; adjuvant tamoxifen was administered from May 2021 to June 2023. In June 2023, imaging identified enlarged pathological axillary and internal mammary nodes as well as supraclavicular bone metastases; bone scan confirmed sternal and osteolytic lesions, and biopsy of a supraclavicular node confirmed metastatic adenocarcinoma with ER 8/8, PR 0/8, HER2 +3. The patient received palliative radiotherapy to the sternal bone, and multidisciplinary discussion recommended taxane-based chemotherapy with trastuzumab and zoledronic acid until maximal response, followed by a shift to an aramidex-based regimen. The patient reported increased sternal pain after discontinuing trastuzumab for a two-month maintenance break; she subsequently received 24 cycles of maintenance trastuzumab and femara. Recent imaging showed multiple enlarged mediastinal lymph nodes; a CT CAP performed in February 2025 demonstrated newly developed subcarinal lymphadenopathy, and the latest CT CAP in April 2026 confirmed these findings. The patient is currently scheduled for trastuzumab (Tykerb) at a schedule of $5 \times 1 \times 30$, with the next clinic visit planned for 14 July 2026. The patient has completed 24 cycles of Herceptin to date.		
تحريرا في ,		
	الوقت :	التاريخ : 19-09-2026
Reham Ahmed Fathy Ibrahim		الطبيب المعالج :
يعتمد ,		

Patient's Name:

SABAH MOUSAAD SELMAN BARSOUM

Date:

2026-04-07

Referred by:

Prof.Dr. CT CAP+C

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported breast carcinoma , no available studies for comparison

(I) Chest:

- ☐ Left mastectomy with clear operative bed , no masses or collection
- ☐ Few enlarged mediastinal lymph nodes , the largest at subcarinal group 9 x 5 mm . .
- ☐ Clear both lung fields .no sizable pulmonary masses or consolidation patches
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous low CT density , no focal lesions
- ☐ Patent and homogenously enhancing portal vein.
- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen .
- ☐ Regular filling of the urinary bladder showing no stones, masses .

Patient's Name: SABAH MOUSAAD SELMAN BARSOUM
Date: 2026-04-07
Referred by: Prof.Dr. CT CAP+C
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported breast carcinoma ,compared to last study dated 5/11/2025 current study;

(I) Chest:

- Left mastectomy with clear operative bed , no masses or collection
- Regressive course as regard the previously noted enlarged mediastinal lymph nodes ,current study reveals the largest at subcarinal group 9 x 5 mm compared to 22 x15 in last study.
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous low CT density , no focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.

Patient's Name: SABAH MOUSAAD SELMAN BARSOUM
Date: 2026-04-07
Referred by: Prof.Dr. CT CAP+C
Hospital: Goustave Roussey International Egypt

- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Stationary course as regard the Sclerosis of the sternum upper body and manubrium .
- No vertebral structural collapse .

OPINION:.

- Generalized stationary to regressive course noted as regard the last study.

Dr Abubakr Hejazi MSc FEBR
Dr.Reham Khedr, MD

Patient Name: Sabah Mossad Soliman Barsoum	Date : 05/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 2717/2023	Pat. ID: 41679

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:

HISTORY : As compared to prior study report dated at 10/7/2025 ,the current study reveals:

1. CHEST:

- Left MRM with no gross enhancing masses.
- few subcentimetric left axillary, internal mammary and supra-clavicular lymph nodes.
- bilateral mosaic attenuation with areas of air trapping
- **Still noted** multiple enlarged prevascular, paratracheal, subcarinal and right hilar mediastinal lymph nodes largest at subcarinal measuring 2.2x 1.5 cm.
- No evidence of pleural effusion, .
- Average cardiac size.
- No pericardial effusion detected.

2. ABDOMEN AND PELVIS:

- **Stationary course** as regards the mildly enlarged liver with **still noted** homogenous low CT attenuation denoting fatty infiltration. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with **no time interval changes as regards** the ill-defined mid-polar small focal lesions measures 7 x 10 mm.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- **Still noted** bulky both suprarenal glands bilaterally..
- Normal appearance of the pancreas, aorta and I.V.C.

Patient Name: Sabah Mossad Soliman Barsoum	Date : 05/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 2717/2023	Pat. ID: 41679

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.

3. The scanned skeleton:

- **Stationary** multiple predominately sclerotic osseous lesions seen at the sternal manubrium and proximal body.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- Follow up study with overall rather stationary course since last study compared to report only.

Dr. Nourhan Mohamed, MD

Dr. Nourhan Mohamed, MD

Signature Dr. Mohamed Hassan
15/11/2025 3:03 AM

Patient Name: Sabah Mossad Soliman Barsoum	Date : 05/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 2717/2023	Pat. ID: 41679

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:

HISTORY : As compared to prior study report dated at 10/7/2025 ,the current study reveals:

1. CHEST:

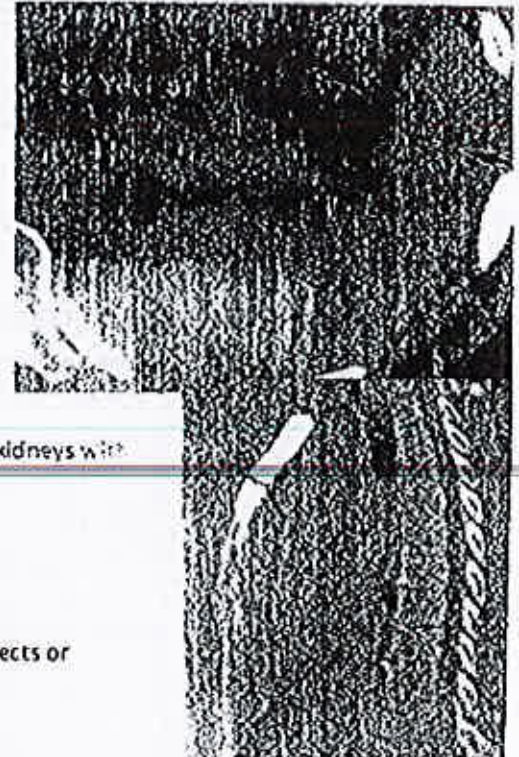
- Left MRM with no gross enhancing masses.
- few subcentimetric left axillary, internal mammary and supra-clavicular lymph nodes.
- bilateral mosaic attenuation with areas of air trapping
- **Still noted** multiple enlarged prevascular, paratracheal, subcarinal and right hilar mediastinal lymph nodes largest at subcarinal measuring 2.2x 1.5 cm.
- No evidence of pleural effusion, .
- Average cardiac size.
- No pericardial effusion detected.

2. ABDOMEN AND PELVIS:

- **Stationary course** as regards the mildly enlarged liver with **still noted** homogenous low CT attenuation denoting fatty infiltration. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with **no time interval changes as regards** the ill-defined mid-polar small focal lesions measures 7 x 10 mm.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- **Still noted** bulky both suprarenal glands bilaterally .
- Normal appearance of the pancreas, aorta and I.V.C.

Patient Name: Sabah Mossad Soliman Barsoum	Date : 10/07/2025
Exam : CT NCAP +C(CT Neck Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 2717/2023	Pat. ID: 41679

- Stationary course as regards the mildly enlarged liver with still noted homogenous low CT attenuation denoting fatty infiltration. No definite focal lesions or contour irregularities.
 - Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Normal size & parenchymal texture of the spleen with no time interval changes as regards the ill-defined mid-polar small focal lesions measures 7 x 10 mm.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Still noted bulky both suprarenal glands bilaterally .
 - Normal appearance of the pancreas, aorta and I.V.C.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Normal appearance of the other pelvic organs.
3. the scanned skeleton:
- Stationary multiple predominately sclerotic osseous lesions seen at the sternal manubrium and proximal body.
 - No vertebral structural collapse or extra-osseous soft tissue component.



OPINION:

- Follow up study with overall stationary course of the disease

Mona Ali, M.D.

Patient Name: Sabah Mossad Soliman Barsoum	Date : 10/07/2025
Exam : CT NCAP +C(CT Neck Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 2717/2023	Pat. ID: 41679

Signature Dr. Mona Ali
17/07/2025 12:05 AM



Patient Name	: الأستاذة / مناج محمد سليمان	Request No.	: 13002590
Patient No.	: 27210090100323	Request Date	: 24/07/2023
Path No.	: SR-808/23	Verification Date	: 31/07/2023
Sex	: Female	Financial Category	: ارقام دة السلام - فئة الدولة
Age	: 51 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Tru cut slide of breast:
Invasive duct carcinoma, Grade II
Tru cut slide of lymph nodes:
Metastatic adenocarcinoma of possible primary mammary ductal origin.
Operation slides:
Invasive duct carcinoma, Grade II with major DCIS component.
Free nipple and areola
Free received margin
Lymph nodes: (1/13) with focal capsular invasion.
IPT:
ER: Positive (score 8/8).
PR: Negative (score 0/8).
HER-2: Positive (score 3+).

قسم الباثولوجي

Prof. Gamal Abdelatty
Dr. Heba Hassan

Verified by :
dr.esraa magdy

Printing Date : 02/08/2023 09:09:35

Page 2 of 2

Printing By : marwa mohamed youss

مركز الباثولوجي - مبنى 10 - طابق 1 - شارع 10 - القاهرة - مصر
Phone No. 02 - Fax No. 02 - Email: info@pibh.gov.eg
Website: www.pibh.gov.eg

ECHO REPORT

Name:

Date: 7 / 9 / 2025

EF	73%	55-75%	PWT	0.8	0.6-1.1cm
LV EDD	4.2	3.5-5.8cm	AO.R	2.6	2-3.7cm
LV ESD	2.2	2-4cm	LA	3.6	2-4cm
FS	35%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.3	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function Normal

SWMA No. RWMA

LA and Ao root dimensions Normal

Mitral Valve

Aortic valve ✓

Tricuspid valve ✓

Pulmonary valve

Right side and systolic function TAPSE: 2.7

Intracardiac masses and effusion

Dr /

Medical No. : 9885
Patient's Name: sabah mosaad soliman barsom
Date: 2026-09-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported breast carcinoma underwent left MRM, no available DICOM images for comparison

(I) Chest:

- Left mastectomy with clear operative bed , no masses or collection
- Few enlarged mediastinal lymph nodes , the largest at subcarinal group 9 x 5 mm . .
- Diffuse mosaic attenuation of both lungs ? small airway disease for clinical and laboratory assessment.
- No sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous low CT density , no focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Diffuse sclerosis of the sternum. D4 vertebral body and iliac bones bilaterally.
- Few tiny scattered vertebral lytic foci.
- No vertebral structural collapse.

OPINION:

- No pulmonary or hepatic metastatic lesions.
- Diffuse sclerosis of the sternum, D4 vertebral body and iliac bones bilaterally with few scattered vertebral lytic foci for bone scan.

Dr.sherif Wagdy